Health Economics Analysis Plan (HEAP) for the MELanoma SELF surveillance (MEL-SELF) trial: a randomised controlled trial of patient-led compared to clinician-led surveillance in people treated for early-stage melanoma.

1 Administrative information

2 Trial introduction and background

3 Economic approach

4 Economic data collection and management

5 Economic data analysis

6 Modelling

7 Reporting and publishing

8 References

9 Appendices

Version 1.0

12 December 2025

1 Administrative information

1.1 Title

Health Economics Analysis Plan (HEAP) for the MELanoma SELF surveillance (MEL-SELF) trial: a randomised controlled trial of patient-led compared to clinician-led surveillance in people treated for early-stage melanoma.

1.2 Trial registration number

Australian New Zealand Clinical Trials Registry (ANZCTR) registration number: ACTRN12621000176864

1.3 Source of funding

The MEL-SELF trial is funded by an Australian National Health and Medical Research Council (NHMRC) Project Grant (APP1163054).

1.4 Purpose of HEAP

The purpose of this HEAP is to describe the analysis and reporting procedure intended for the economic evaluation of the MEL-SELF trial. The HEAP is designed to ensure there is no conflict with the protocol and associated statistical analysis plan (SAP) and it should be in conjunction with them.

1.5 Trial protocol version

MEL-SELF trial protocol, version 9, dated 4 December 2024.

1.6 Trial SAP version

MEL-SELF trial SAP, described elsewhere.[1]

1.7 Trial HEAP version

MEL-SELF trial HEAP, version 1, dated 12 December 2025.

1.8 HEAP revisions

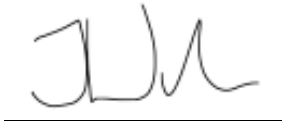
See Table 1.

1.9 Roles and responsibilities

This HEAP was prepared by Mr Jake T W Williams and approved by Prof Rachael L Morton (senior health economist) and Prof Katy J L Bell (MEL-SELF trial Chief Investigator). Prof Rachael Morton has overall responsibility for the production and reporting of the results of the economic evaluation. Jake Williams, or other persons performing the economic analyses, is responsible for ensuring the results of this evaluation are accurate and consistent with other trial analyses. Differences will be

discussed with Prof Katy Bell and the MEL-SELF trial statisticians prior to being reported.

1.10 Signature of person writing HEAP



12 December 2025

Mr Jake T W Williams

1.11 Signature of senior health economist



12 December 2025

Prof Rachael L Morton

1.12 Signature of Chief Investigator



12 December 2025

Prof Katy J L Bell

Table 1. Revisions to the MEL-SELF Trial HEAP

Protocol version	Updated HEAP version	Section number changed	Description and reason for change	Individual making the change	Date changed
9	1	-	-	-	12 December

Table of Contents

1 Administrative information 2

1.1 Title 2

1.2 Trial registration number 2

1.3 Source of funding 2

1.4 Purpose of HEAP 2

1.5 Trial protocol version 2

1.6 Trial SAP version 2

1.7 Trial HEAP version 2

1.8 HEAP revisions 2

1.9 Roles and responsibilities 2

1.10 Signature of person writing HEAP 3

1.11 Signature of senior health economist 3

1.12 Signature of Chief Investigator 3

2 Trial introduction and background 8

2.1 Trial background and rationale 8

2.2 Aims of the trial 8

2.3 Objectives 8

2.4 Trial population 9

2.5 Intervention and control 9

2.6 Trial design 10

2.7 Trial start and end dates 10

3 Economic approach 11

3.1 Aim of the economic evaluation 11

3.2 Objectives of economic evaluation 11

3.3 Overview of economic analysis 11

3.4 Jurisdiction 11

3.5 Perspective 11

3.6 Time horizon 11

4 Economic data collection and management 13

4.1 Monitoring collection of health economic data 13

4.2 Database management 13

4.3 Data entry 13

4.4 Data archiving 13

4.5 Statistical software 13

4.6 Identification of resources 13

4.7 Measurement of resource-use data 14
4.8 Valuation of resource-use data 14
4.9 Environmental impact of resource-use data 14
4.10 Identification of outcomes 15
4.11 Measurement of outcomes 15
4.12 Valuation of outcomes 15

5 Economic data analysis 16

5.1 Analysis population 16
5.2 Timing of analyses 16
5.3 Discount rates 16
5.4 Cost-effectiveness threshold 16
5.5 Statistical decision rule 16
5.6 Analysis of resource use 16
5.7 Analysis of costs 16
5.8 Analysis of environmental impact 17
5.9 Analysis of outcomes 17
5.10 Data cleaning for analysis 17
5.11 Missing data 17
5.12 Analysis of cost-effectiveness 17
5.13 Sampling uncertainty 18
5.14 Subgroup analysis 18
5.15 Sensitivity analysis 18

6 Modelling 19

7 Reporting and publishing 20

7.1 Reporting standards 20
7.2 Reporting deviations from the HEAP 20

8 References 21

9 Appendices 22

Table 2. Abbreviations

AJCC	American Joint Committee on Cancer
ANZCTR	Australian New Zealand Clinical Trials Registry
ASICA	Achieving Self-directed Integrated Cancer Aftercare
CCA	Cost-consequence analysis
CEA	Cost-effectiveness analysis
DASS	Depression, anxiety, and stress scales
FCR	Fear of cancer recurrence
FCRI	Fear of cancer recurrence inventory
HEAP	Health Economic Analysis Plan
ICER	Incremental cost-effectiveness ratio
MBS	Medicare Benefits Schedule
MEL-SELF	MELanoma SELF surveillance trial: a randomised controlled trial of patient-led compared to clinician-led surveillance in people treated for early-stage melanoma.
NHMRC	National Health and Medical Research Council
PBS	Pharmaceutical Benefits Scheme
RCT	Randomised controlled trial
SAP	Statistical Analysis Plan
SSE	Skin self-examination

2 Trial introduction and background

2.1 Trial background and rationale

After receiving curative surgical treatment for localised melanoma (American Joint Committee on Cancer [AJCC] stage 0-II) patients are at increased risk of a new primary melanoma or a keratinocyte skin cancer, as well as being at risk of a recurrence of the index melanoma.[2] Follow-up care involves surveillance through full body skin examinations to facilitate early detection and treatment of new or recurrent melanoma or keratinocyte cancer.

The current model of follow-up care is clinician-led surveillance. This involves routinely scheduled clinic visits with a doctor (e.g., skin cancer trained general practitioner, or dermatologist) to examine skin lesions, with additional appointments arranged to investigate any patient-detected changes as needed. Theoretical benefits from clinician-led surveillance from earlier detection and treatment currently lack robust empirical evidence from randomised trials. There is evidence that clinic visits are associated with health system and out-of-pocket costs, opportunity costs, psychological harms, and carbon emissions.[3,4]

Patient-led surveillance is a new model of care that involves patients taking a greater role in the management of their post-treatment follow up. With patient-led surveillance, patients are supported to undertake skin self-examinations (SSE) at home and may receive dermatologist opinion on any concerning lesions using smartphone dermatoscope devices and teledermatology. If the dermatologists assesses that clinical review is needed, then the patient is provided fast-tracked access to an unscheduled clinic visit with their usual melanoma doctor. Routinely scheduled appointments are maintained but may occur less frequently if the doctor and patient decide that this is appropriate. This model of care has the potential to reduce the number of routinely scheduled clinic visits required, while maintaining (or even increasing) potential benefits of earlier detection and treatment of new and recurrent skin cancers.

The pilot MELanoma SELF surveillance (MEL-SELF) Randomised Controlled Trial (RCT; n=100) demonstrated the feasibility and acceptability of a patient-led surveillance intervention that incorporated The Achieving Self-directed Integrated Cancer Aftercare (ASICA) Skin Checker app and patient-performed mobile teledermoscopy.[5,6] The clinical effectiveness is being evaluated in the definitive MEL-SELF RCT.[1,7]

2.2 Aims of the trial

The definitive MEL-SELF trial aims to compare the effects of patient-led surveillance and clinician-led surveillance on health outcomes, psychological outcomes, and resource use. It aims to test the hypotheses that patient-led surveillance results in better health and psychological outcomes than clinician-led surveillance and consumes fewer health resources.

2.3 Objectives

The primary objective of the MEL-SELF trial is to assess whether patient-led surveillance compared to clinician-led surveillance increases the proportion of participants who are diagnosed with a new primary melanoma ahead of a routinely scheduled visit (via a fast-tracked unscheduled clinic visit).

The secondary objectives are to assess the impact of patient-led surveillance compared to clinician-led surveillance on: the time to diagnosis of any skin cancer; skin-self-examination beliefs and practice; psychological outcomes; patient self-efficacy and social support; the acceptability of a hypothetical reduction in routinely scheduled clinic visit frequency; and healthcare resource use and cost. It will also assess the acceptability of patient-led surveillance to patients and clinicians, and evaluate the technical performance of two different models of dermatoscope.

2.4 Trial population

The inclusion criteria for participants entering the MEL-SELF trial are that they:

- Have completed treatment for stage 0/I/II melanoma, and are attending regular melanoma follow-up as indicated by at least one scheduled visit within the next 12 months at a recruiting clinic,
- Can undertake SSE,
- Have a suitable study partner (spouse, partner, family member, friend) to help with self-examination and imaging,
- Own a smartphone (and have access to Internet / email / SMS text messaging),
- Have a scheduled visit at a recruiting clinic not more frequently than 6 monthly
- Are able to give informed consent,
- Have sufficient English language skills to read the materials and complete the questionnaires, and
- Are aged 18 years or older.

The exclusion criteria for the MEL-SELF Trial are:

- Have ever had stage III/IV melanoma
- Have a known past or current diagnosis of cognitive impairment
- Were participants in the MEL-SELF pilot trial (conducted Nov 2018 - Feb 2020).
- Have a smartphone that is not compatible with the mobile dermatoscopes that are part of the intervention.

2.5 Intervention and control

In the control arm, patients receive clinician-led surveillance (usual care), which includes routinely scheduled visits to their treating clinician, unscheduled visits as needed, and an educational booklet titled ‘Your guide to early Melanoma’.

In the intervention arm, patients receive patient-led surveillance, which includes usual care plus a mobile dermatoscope, a smartphone application for store-and-forward

teledermatology, fast-tracked unscheduled clinic visits if recommended by the teledermatologist, instructions and training on how to use the dermatoscope and teledermatology, access to video guidance on how to complete total skin self-examination (SSE), reminders to complete SSE.

2.6 Trial design

The MEL-SELF trial is a two-arm, parallel, superiority, randomised controlled trial (RCT) with 500 participants with a 1:1 allocation ratio between the intervention and control groups. A second randomised step within the intervention arm will randomise participants 1:1 into alternative models of dermatoscope (polarised or non-polarised light source).

2.7 Trial start and end dates

The first patient was randomised into the MEL-SELF trial on 16 December 2021. The last patient was randomised on 3 June 2024. The follow up period will run for two years.

3 Economic approach

3.1 Aim of the economic evaluation

The aim of this economic evaluation is to perform a cost-consequence analysis and quantify the health outcomes, costs, and environmental impacts of patient-led surveillance compared with clinician-led surveillance as a mode of follow up for patients treated for an early-stage melanoma.

3.2 Objectives of economic evaluation

We will quantify the health outcomes, health system costs, out-of-pocket costs, productivity changes, and carbon emissions of patient-led surveillance compared to clinician-led surveillance over a period of 24-months from the health system and societal perspectives. We will also compare two types of dermatoscope (polarised or non-polarised light source) used by patients receiving patient-led surveillance.

3.3 Overview of economic analysis

The within-trial economic evaluation will be performed using individual patient-level data from the MEL-SELF trial. These data will be linked to the New South Wales Cancer Registry and the Medicare Benefits Schedule (MBS). Disaggregated resource use, healthcare costs, environmental impacts, and health impacts will be presented as the mean costs per participant over the 24-month study period, by randomised treatment allocation. Mean health impacts will be estimated in consultation with the MEL-SELF trial statistician. Environmental impacts will be reported using a parallel approach and not be valued in health or monetary units or included in any summary ratios.

3.4 Jurisdiction

The MEL-SELF trial is being conducted in Australia, which has a government-funded health system and provides subsidised access to medical care at the point-of-use. Most health system costs are incurred primarily though the Medicare Benefits Schedule (MBS) and Pharmaceutical Benefits Scheme (PBS), programmes that subsidise common healthcare activities and prescribed medicines. Private healthcare is also available for those with private health insurance.

3.5 Perspective

The base case analysis will be conducted from the health system perspective. A secondary analysis will be conducted from the societal perspective and include health system costs, patient out-of-pocket costs, productivity changes, and environmental impacts.

3.6 Time horizon

The within-trial economic evaluation will be conducted over a period of two years, using data collected from randomisation to the end of the study follow up period. The time

horizon of environmental impacts used in the study will be reported in an appendix to the manuscript.

4 Economic data collection and management

4.1 Monitoring collection of health economic data

Data was collected from patient clinic records and monthly retrospective self-reported resource use electronic health diaries. These were sent to participants at the start of each month for the first 12 months post-randomisation, to report on the month immediately prior. Participants were reminded automatically via SMS and email to complete the resource use diaries. MEL-SELF Trial staff called participants to remind them to complete the resource use diaries, and to resolve any inquiries that arose during data collection. The MEL-SELF trial Database Manager monitored the collection of resource use diaries and clinic records for the duration of the study.

4.2 Database management

Trial data, including all economic data, will be securely stored on the trial database on REDCap (Vanderbilt University: Nashville, USA) and managed by the MEL-SELF trial Database Manager.

4.3 Data entry

Doctor's letters, pathology reports, and relevant clinical notes will be identified from the patient's electronic medical record by MEL-SELF Trial Site Coordinators. These documents will be redacted and saved to the trial database. Key variables will be extracted and entered directly into the database. Trial participants will complete the resource use diary via REDCap and this will automatically save to the trial database. Data checks will be undertaken by the Database Manager.

4.4 Data archiving

A copy of the health economic analysis files, derived datasets, interim datasets, and final analysis will be locked and archived. Archived digital datasets will be held in the NHMRC Clinical Trials Centre, University of Sydney on a secure server, and will conform to the department's data security policy and department data compliance and Data Protection Act policies.

4.5 Statistical software

R version 4.4.3 (R Foundation for Statistical Computing: Vienna, Austria) and R Studio version 2025.05.0+496 (Posit Software: Boston, USA), or higher, will be used for all analyses described in this document.

4.6 Identification of resources

From the health system perspective, the following resources will be identified for inclusion in the economic evaluation: teledermatology; scheduled and unscheduled visits to the patient's dermatology clinic; general practitioner, specialist, and other allied health professional visits; surgical treatments (e.g., excisional biopsies, wide

excisions), diagnostic tests and scans (e.g., blood tests, CT scan, ultrasound), and other relevant procedures; hospital admissions and emergency department presentations.

For the secondary analysis from the societal perspective, we will also identify and include: patient direct out-of-pocket costs for melanoma-related items (e.g., sunscreen, hats) and for healthcare use (e.g., parking costs, gap payments); patient time off work, time organising healthcare appointments, and time receiving care from friends and family.

Resources related to the delivery of the intervention itself will not be included, such as the cost of mobile dermatoscope, and providing training to patients on skin self-examination.

4.7 Measurement of resource-use data

A monthly self-reported retrospective healthcare use diary was completed by trial participants up to 12-months post-randomisation. Clinic records from the treating melanoma clinic were collected up to 24-months post randomisation. Data linkage to MBS claims will take place at the end of the trial follow up period for data up to 24-months post randomisation. All three sources will be used for the measurement of healthcare resources. Episodes of teledermatology, healthcare appointments, treatments, tests, procedures, and emergency department visits will be measured by volume and type. Hospital admissions will be measured by length of stay in days. Time off work, organising appointments, and receiving care will be measured in hours. Out-of-pocket costs will be measured directly by the type of item.

4.8 Valuation of resource-use data

Healthcare resources will be valued using appropriate Australian specific unit costs in Australian dollars. Teledermatology, healthcare appointments, treatments, tests, and procedures will be valued using Medicare Benefits Schedule (MBS) item numbers at 85% benefit amounts. Hospital admissions will be valued using Australian Refined Diagnosis Related Group (AR-DRG), National Weighted Activity Units (NWAU) and the most recently available published national efficient price. Costs will be adjusted for inflation, as necessary, to the appropriate reference year using the Australian Institute of Health and Wellbeing total Health Price Index. Patient out-of-pocket expenses will be valued using self-reported costs (e.g., for consumables like sunscreen, hat) and estimated by calculating the difference between the Medicare reimbursement and actual charges from participating clinics. Time will be valued using the human capital approach using known wages or estimated wages.

4.9 Environmental impact of resource-use data

[5.9] Carbon emissions will be included in kg carbon dioxide equivalent units (CO_2-e). Other environmental impacts will not be considered. The carbon emissions of all resources identified in section 5.6 will be included by considering the carbon emission of the resource itself and of patient travel. Emissions from services will be identified from existing process-based life cycle assessments, such as those listed in the

HealthcareLCA database (www.healthcarelca.com) or Lancet MedZero database (https://www.medzerocarbon.com). The quality of estimates will be interrogated before using them. Where existing high-quality data are not available assumptions will be made and documented based on similar healthcare resources. Emissions from patient transport will be included based on travel distance and mode of transport (e.g., walk, car, train etc) information recorded in the monthly healthcare use diary. Emissions factors for transport will be obtained from the Australian National Greenhouse Accounts Factors and EcoInvent. Environmental impacts will not be valued in monetary units.

4.10 Identification of outcomes

Two outcomes will be included. First, we will include the proportion of patients with a new primary or recurrent melanoma at a fast-tracked unscheduled visit. This outcome is favourable, representing a proxy for the early detection of melanoma. Second, we will include the number of cases of clinically significant fear of cancer recurrence avoided.

4.11 Measurement of outcomes

The number of new primary and recurrent melanomas diagnosed at an unscheduled visit will be obtained from clinic records (pathology reports) and confirmed by the MEL-SELF trial dermatopathologist. Classification of an appointment as unscheduled will be based on participant's clinic records (doctor's letters) and will be undertaken by MEL-SELF endpoint adjudication committee. Fear of cancer recurrence (FCR) will be measured using a melanoma-specific version of the 9-item Fear of Cancer Recurrence Inventory (FCRI) severity subscale, where a higher score indicated greater levels of FCR. FCRI scores will be measured at baseline, 6-months, and 12-months. A score ≥13 on the FCRI severity subscale will be considered clinically significant.

4.12 Valuation of outcomes

Outcomes will be valued in natural units as described in section 5.11.

5 Economic data analysis

5.1 Analysis population

The economic evaluation will adhere to the intention-to-treat (ITT) principle and analyse all participants according to their randomly assigned study arm (i.e., patient-led, or clinician-led surveillance) regardless of their adherence to the protocol. Sensitivity analyses may be conducted using per-protocol or as-treated principles.

5.2 Timing of analyses

The primary analysis will be conducted once all trial participants have completed follow up of 24-months post-randomisation.

5.3 Discount rates

A discount rate of 5% will be used for costs and health outcomes in the second year of the study, consistent with recommendations by the Australian Pharmaceutical Benefits Advisory Committee (PBAC) and Medical Services Advisory Committee (MSAC). Carbon emissions (in kg CO_2-e) will not be discounted in this study, to ensure intergenerational equity. This is consistent with the approach recommended by McAlister et al.[8]

5.4 Cost-effectiveness threshold

No cost-effectiveness threshold will be used (as per the cost-consequence study design), and all costs, outcomes, and environmental impacts will be reported in a disaggregated format.

5.5 Statistical decision rule

Mean group differences will be interpreted as significant if the 95% confidence intervals between groups have no overlap. Appropriate significance testing may be performed, and differences will be considered significant if calculated p-values are less than 0.05.

5.6 Analysis of resource use

Mean and standard deviations (SD) of resource use will be reported by key categories for each treatment group. Group differences will be reported with 95% confidence intervals.

5.7 Analysis of costs

Mean per-patient costs and standard deviations (SD) will be summarised for each treatment group. Participants in the MEL-SELF trial were randomised using minimisation based on key prognostic variables (melanoma stage, age group, sex, clinic type [specialist vs GP-led], risk of subsequent primary melanoma, and presence of dysplastic naevus syndrome). Differences in total mean costs between the intervention and control arms will first be calculated with 95% confidence intervals, estimated using

bootstrapping with 10,000 replications to assess uncertainty in differences in costs. Then, ordinary least squares (OLS) regression will be used to adjust for these minimisation variables. Generalised linear models (GLM) may be used instead to adjust for participants characteristics and skewness of cost data. Group differences will be presented with

5.8 Analysis of environmental impact

Mean per-patient carbon emissions and standard deviations will be summarised for each treatment group. Mean difference between the intervention and control arms will be calculated with 95% confidence intervals.

5.9 Analysis of outcomes

The proportion of patients who have a melanoma diagnosed at an unscheduled clinic visit and clinically significant fear of cancer recurrence will be summarised for each treatment group. Logistic regression will be used to estimate the between group difference in the proportion of participants with each outcome. An adjusted regression analysis will also be conducted to account for participant age, sex, whether the recruiting clinic is a GP or specialist clinic, melanoma substage, and subsequent new primary melanoma risk score. Group differences will be presented with 95% confidence intervals. Details of the statistical analysis are reported in the MEL-SELF statistical analysis plan (SAP).

5.10 Data cleaning for analysis

Data cleaning will be conducted in R and all corrections to the data will be noted in the R code. Face validity tests will be conducted to clean data by identifying likely errors, misspelt variables, and logical inconsistencies. Any errors or unlikely values will be removed.

5.11 Missing data

Preference will be given to linked Medicare data, and this will be supplemented by information from patient healthcare diaries and clinic records. Missing data is possible due to patient withdrawal, non-response, or loss to follow-up. Descriptive statistics will be used to explore the amount of missing data, patterns in missing data, and the association between missingness, baseline variables, and outcomes. If appropriate, the base case analysis will assume that data is missing at random (MAR). Multiple imputation methods will be used for cost and environmental impact data. Retrieved dropout multiple imputation or mixed model will be used for health outcomes, consistent with the MEL-SELF SAP. Sensitivity analyses will be conducted to explore departures from the MAR assumption (or other assumed methods), and this may include methods such as reference-based multiple imputation.

5.12 Analysis of cost-effectiveness

The outcomes, costs, and environmental impacts will be reported in disaggregated formats. Tables will present summary data by treatment group.

5.13 Sampling uncertainty

NA

5.14 Subgroup analysis

Analysis will also be conducted to investigate how outcomes, costs, carbon emissions, vary between different patient subgroups. Subgroup analyses may include sociodemographic (e.g., age, sex, socioeconomic status, remoteness) and clinical variables (e.g., treatment centre, melanoma substage, dysplastic naevus syndrome, risk of subsequent new primary melanoma). Subgroups may be omitted from the analysis based on their sample size.

5.15 Sensitivity analysis

Sensitivity analyses will be undertaken to explore uncertainties surrounding key parameters in the economic evaluation. A sensitivity analysis will be conducted to explore assumptions about missing data. A sensitivity analysis will be conducted that applies a 5% p.a. discount to environmental impact.

6 Modelling

NA

7 Reporting and publishing

7.1 Reporting standards

Consolidated Health Economic Evaluation Reporting Standards (CHEERS) guidelines will be followed when reporting the health economic evaluation. This work will be reported in at least one publication. Additional publications will be considered if appropriate.

7.2 Reporting deviations from the HEAP

Any deviations from this document that occur in the economic analysis will be documented. These deviations will be included as an appendix to the published analysis.

8 References

1 Medcalf E, Taylor A, Turner R, Espinoza D, Bell KJL. Can patient-led surveillance detect subsequent new primary or recurrent melanomas and reduce the need for routinely scheduled follow up? Statistical analysis plan for the MEL-SELF randomised controlled trial. *Contemporary Clinical Trials* 2022; **117**: 106761.

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3 Williams JTW, Dieng M, Bell K, McAlister S, Morton RL. Healthcare Costs and Carbon Emissions of Stage III Melanoma Surveillance Imaging. *Appl Health Econ Health Policy* 2025; published online Aug 30. DOI:10.1007/s40258-025-00998-1.

4 Williams JTW, Bell K, Morton RL, Guitera P, Martin L, McAlister S. The climate change impact of surgical and non-surgical skin cancer appointments: a process-based life cycle assessment. *Open Science Framework* 2025; published online June 30. DOI:10.17605/OSF.IO/E8CX2.

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7 Ackermann DM, Smit AK, Janda M, *et al.* Can patient-led surveillance detect subsequent new primary or recurrent melanomas and reduce the need for routinely scheduled follow-up? A protocol for the MEL-SELF randomised controlled trial. *Trials* 2021; **22**: 324.

8 McAlister S, Morton RL, Barratt A. Incorporating carbon into health care: adding carbon emissions to health technology assessments. *The Lancet Planetary Health* 2022; **6**: e993–9.

9 Appendices

Appendix 1. Web-based healthcare resource use diary

Monthly resource use diary

Please complete this diary as soon as possible after being invited to complete it. The questions aim to find out the financial costs of your melanoma to you, your family and the health system every month. This will help us understand the financial costs of melanoma and may help improve services for people with melanoma. If you are unsure about any answers, please write down your best guess.

Thank you very much for your assistance with this trial.

Please select the last complete calendar month e.g., if it is now November, you should select October	◯ January ◯ February ◯ March ◯ April ◯ May ◯ June ◯ July ◯ August ◯ September ◯ October ◯ November ◯ December
Please select the year	◯ 2021 ◯ 2022 ◯ 2023 ◯ 2024
Please select the category that best describes your current employment	◯ In full-time employment (30 hours or more per week) ◯ In part-time employment (less than 30 hours per week) ◯ Employed but currently on sick leave ◯ Retired ◯ Not in paid employment
Approximately how many hours have you taken off work in total during [event-name][month] due to your melanoma or other skin cancer?	_____ (hours)
Have you received help or support from family or friends due to your melanoma or other skin cancer during [event-name][month]? (e.g., help check your skin or drive you to your melanoma appointment)	◯ Yes ◯ No
Approximately how many hours have family and friends helped you due to your melanoma or other skin cancer during [event-name][month]?	_____ (hours)

Healthcare Use

Did you use any healthcare services because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?	◯ Yes ◯ No (This includes visits to your local GP or specialist, diagnostic tests, hospital stays, any money spent on medical care, goods or supplies etc.)
Clinic Visits	
Did you attend any clinic visits because of your melanoma or other skin cancer during [event-name][month]?	◯ Yes ◯ No (This includes attendance at [preenrolment_arm_1][formen_clinic] and any other clinic related to your melanoma, e.g., your local GP)
Were any of these clinic visits at [preenrolment_arm_1][formen_clinic]?	◯ Yes ◯ No
How many routine follow-up clinic visits at [preenrolment_arm_1][formen_clinic] did you attend because of your melanoma or other skin cancer during [event-name][month]?	_____
Please select the date of routine visit 1	_____
Please select the date of routine visit 2	_____
How many minutes in total did you spend organising routine follow up clinic visit because of your melanoma or other skin cancer during [event-name][month]? You can include the time spent directly in communication with clinic staff as well as the time spent organising other responsibilities (e.g., time off work, childcare or carer-duties) that enable you to attend the follow-up visit(s).	_____

Did you use any of the following modes of transport to get to your routine visit(s)? (please select all that apply)

Routine visit 1	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____

Routine visit 2

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Approximately how much money did you spend on car parking for these visit(s)?

Parking Costs ($): Routine visit 1

_____
($)

Parking Costs ($): Routine visit 2

_____
($)

Approximately how much money did you spend on public transport for these visit(s)?

Public Transport Costs ($): Routine visit 1

_____
($)

Public Transport Costs ($): Routine visit 2

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a routine clinic visit in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Routine visit 1

_____

Finish Place: Routine visit 1

_____

Start Place: Routine visit 2

_____

Finish Place: Routine visit 2

_____

Powered by REDCap

How many short notice urgent clinic visits at [preenrolment_arm_1][formen_clinic] did you attend because of your melanoma or other skin cancer during [event-name][month]?

You can include the time spent directly in communication with clinic staff as well as the time spent organising other responsibilities (e.g., time off work, childcare or carer-duties) that enable you to attend the follow-up visit(s).

_____

Please select the date of urgent visit 1

_____

Please select the date of urgent visit 2

_____

How many minutes in total did you spend organising short notice urgent clinic visits because of your melanoma or other skin cancer during [event-name][month]?

_____

Did you use any of the following modes of transport to get to your urgent visit(s)? (please select all that apply)

Urgent visit 1

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Urgent visit 2

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Approximately how much money did you spend on car parking for these visit(s)?

Parking Costs ($): Urgent visit 1

_____
($)

Parking Costs ($): Urgent visit 2

_____
($)

Approximately how much money did you spend on public transport for these visit(s)?

Public Transport Costs ($): Urgent visit 1

_____
($)

Public Transport Costs ($): Urgent visit 2

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a routine clinic visit in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Urgent visit 1

_____

Finish Place: Urgent visit 1

_____

Start Place: Urgent visit 2

_____

Finish Place: Urgent visit 2

_____

Local GP Visits

Did you see your local GP because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

- ◯ Yes
- ◯ No

How many times did you visit your local GP because of treatment for your melanoma or other skin cancer during [event-name][month]?

_____

Please select the date of GP visit 1

_____

Please select the date of GP visit 2

_____

Please select the date of GP visit 3

_____

Did you use any of the following modes of transport to get to your GP visit(s)? (please select all that apply)

GP visit 1

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

GP visit 2

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

GP visit 3

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Approximately how much money did you spend on car parking for these visits during [event-name][month]?

Parking Costs ($): GP visit 1

_____
($)

Parking Costs ($): GP visit 2

_____
($)

Parking Costs ($): GP visit 3

_____
($)

Approximately how much money did you spend on public transport for these visits during [event-name][month]?

Public Transport Costs ($): GP visit 1

_____
($)

Public Transport Costs ($): GP visit 2

_____
($)

Public Transport Costs ($): GP visit 3

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a local GP visit in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: GP visit 1

_____

Finish Place: GP visit 1	_____
Start Place: GP visit 2	_____
Finish Place: GP visit 2	_____
Start Place: GP visit 3	_____
Finish Place: GP visit 3	_____

Specialist Visits not at [preenrolment_arm_1][formen_clinic]

Did you see a specialist doctor (e.g., dermatologist, surgeon) who is not at [preenrolment_arm_1][formen_clinic], because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?	◯ Yes ◯ No
How many times did you visit a specialist doctor because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?	_____
Please select the date of specialist visit 1	_____
Please select the date of specialist visit 2	_____
Please select the date of specialist visit 3	_____
Please select the date of specialist visit 4	_____

Did you use any of the following modes of transport to get to your specialist visit(s)? (please select all that apply)

Specialist visit 1	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____

Specialist visit 2	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Specialist visit 3	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Specialist visit 4	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____

Approximately how much money did you spend on car parking for these visits during [event-name][month]?

Parking Costs ($): Specialist visit 1	_____ ($)
Parking Costs ($): specialist visit 2	_____ ($)
Parking Costs ($): specialist visit 3	_____ ($)
Parking Costs ($): specialist visit 4	_____ ($)

Approximately how money did you spend on public transport for these visits during [event-name][month]?

Public Transport Costs ($): Specialist visit 1	_____ ($)
Public Transport Costs ($): specialist visit 2	_____ ($)

Public Transport Costs ($): specialist visit 3

_____
($)

Public Transport Costs ($): specialist visit 4

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a specialist visit in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Specialist visit 1

_____

Finish Place: Specialist visit 1

_____

Start Place: Specialist visit 2

_____

Finish Place: Specialist visit 2

_____

Start Place: Specialist visit 3

_____

Finish Place: Specialist visit 3

_____

Start Place: Specialist visit 4

_____

Finish Place: Specialist visit 4

_____

Nurse Visits

Did you visit a nurse because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

◯ Yes
◯ No

How many times did you visit a nurse because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

_____

Please select the date of nurse visit 1

_____

Please select the date of nurse visit 2

_____

Did you use any of the following modes of transport to get to your nurse visit(s)? (please select all that apply)

Nurse visit 1

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Nurse visit 2

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Approximately how much money did you spend on car parking for these visit(s) during [event-name][month]?

Parking Costs ($): Nurse visit 1

_____
($)

Parking Costs($): Nurse visit 2

_____
($)

Approximately how much money did you spend on public transport for these visit(s) during [event-name][month]?

Public Transport Costs ($): Nurse visit 1

_____
($)

Public Transport Costs($): Nurse visit 2

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a nurse visit in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Nurse visit 1

_____

Finish Place: Nurse visit 1

_____

Start Place: Nurse visit 2

_____

Finish Place: Nurse visit 2

_____

Other Health Professional Visits

Did you see any other type of health professional (e.g., psychologist, counsellor) because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

- ◯ Yes
- ◯ No

How many times did you visit other health professionals because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

_____

Please select the date of health visit 1

_____

Please select the date of health visit 2

_____

Please select the date of health visit 3

_____

What type of health professional did you see at these visit(s)?

Health visit 1

- ◯ Psychologist
- ◯ Physiotherapist
- ◯ Occupational Therapist
- ◯ Dietitian
- ◯ Other

Please specify

_____

Health visit 2

- ◯ Psychologist
- ◯ Physiotherapist
- ◯ Occupational Therapist
- ◯ Dietitian
- ◯ Other

Please specify

_____

Health visit 3

- ◯ Psychologist
- ◯ Physiotherapist
- ◯ Occupational Therapist
- ◯ Dietitian
- ◯ Other

Please specify

_____

Did you use any of the following modes of transport to get to your other health professional visit(s)? (please select all that apply)

Health visit 1	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Health visit 2	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Health visit 3	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____

Approximately how much money did you spend on car parking for these visit(s) during [event-name][month]?

Parking Costs ($): Health visit 1	_____ ($)
Parking Costs ($): Health visit 2	_____ ($)
Parking Costs ($): Health visit 3	_____ ($)

Approximately how much money did you spend on public transport for these visit(s) during [event-name][month]?

Public Transport Costs ($): Health visit 1	_____ ($)
Public Transport Costs ($): Health visit 2	_____ ($)
Public Transport Costs ($): Health visit 3	_____ ($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a health professional in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Health visit 1

_____

Finish Place: Health visit 1

_____

Start Place: Health visit 2

_____

Finish Place: Health visit 2

_____

Start Place: Health visit 3

_____

Finish Place: Health visit 3

_____

Emergency Department Visits

Did you visit the Emergency Department because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

- ◯ Yes
- ◯ No

How many times did you visit the Emergency Department because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

_____

Please select the date of emergency visit 1

_____

Please select the date of emergency visit 2

_____

Did you use any of the following modes of transport to get to your Emergency Department visit(s)? (please select all that apply)

Emergency visit 1

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Emergency visit 2	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Approximately how much money did you spend on car parking for these visit(s) during [event-name][month]?	
Parking Costs ($): Emergency visit 1	_____ ($)
Parking Costs ($): Emergency visit 2	_____ ($)
Approximately how much money did you spend on public transport for these visit(s) during [event-name][month]?	
Public Transport Costs ($): Emergency visit 1	_____ ($)
Public Transport Costs ($): Emergency visit 2	_____ ($)
Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s). (For example, if you drove from your home in Belrose to a hospital in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)	
Start Place: Emergency visit 1	_____
Finish Place: Emergency visit 1	_____
Start Place: Emergency visit 2	_____
Finish Place: Emergency visit 2	_____
Hospital Admissions	
Were you admitted to the hospital because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?	◯ Yes ◯ No
How many times were you admitted to the hospital because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?	_____

Please select the date of hospital admission 1

_____

Please select the date of hospital admission 2

_____

How many nights did you stay in the hospital?

Hospital admission 1

_____

Hospital admission 2

_____

Did you use any of the following modes of transport to get to your hospital visit(s)? (please select all that apply)

Hospital admission 1

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Hospital admission 2

- ☐ Car
- ☐ Bus
- ☐ Train
- ☐ Bike
- ☐ Walking
- ☐ Other

Please specify

_____

Approximately how much money did you spend on car parking for these visit(s) during [event-name][month]?

Parking Costs ($): Hospital admission 1

_____
($)

Parking Costs ($): Hospital admission 2

_____
($)

Approximately how much money did you spend on public transport for these visit(s) during [event-name][month]?

Public Transport Costs ($): Hospital admission 1

_____
($)

Public Transport Costs ($): Hospital admission 2

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a hospital in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Hospital admission 1 _____

Finish Place: Hospital admission 1 _____

Start Place: Hospital admission 2 _____

Finish Place: Hospital admission 2 _____

Melanoma and other skin cancer related procedures

Did you have any melanoma or other skin cancer related procedures (e.g., biopsy procedures, skin cut-off or skin burnt off...) during [event-name][month]?
- ◯ Yes
- ◯ No

(e.g., biopsy procedures, skin cut-off or skin burnt off)

How many times did you have melanoma or other skin cancer related procedures during [event-name][month]? _____

Please select the date of procedure 1 _____

Please select the date of procedure 2 _____

Please select the date of procedure 3 _____

Please select the date of procedure 4 _____

What type of melanoma or other skin cancer related procedure did you have at these visit(s)?

Procedure 1
- ◯ Skin biopsy/excision
- ◯ Surgery requiring sedation
- ◯ Cryotherapy (skin lesion frozen off with liquid nitrogen)
- ◯ Topical cream therapy (cream applied to the skin lesion)
- ◯ Curettage and cautery (skin lesion scraped off and heat is applied to the skin)
- ◯ Other

Please specify _____

Procedure 2	◯ Skin biopsy/excision ◯ Surgery requiring sedation ◯ Cryotherapy (skin lesion frozen off with liquid nitrogen) ◯ Topical cream therapy (cream applied to the skin lesion) ◯ Curettage and cautery (skin lesion scraped off and heat is applied to the skin) ◯ Other
Please specify	_____
Procedure 3	◯ Skin biopsy/excision ◯ Surgery requiring sedation ◯ Cryotherapy (skin lesion frozen off with liquid nitrogen) ◯ Topical cream therapy (cream applied to the skin lesion) ◯ Curettage and cautery (skin lesion scraped off and heat is applied to the skin) ◯ Other
Please specify	_____
Procedure 4	◯ Skin biopsy/excision ◯ Surgery requiring sedation ◯ Cryotherapy (skin lesion frozen off with liquid nitrogen) ◯ Topical cream therapy (cream applied to the skin lesion) ◯ Curettage and cautery (skin lesion scraped off and heat is applied to the skin) ◯ Other
Please specify	_____

Did you use any of the following modes of transport to get to your melanoma or other skin cancer related procedure(s)? (please select all that apply)

Procedure 1	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Procedure 2	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other

Please specify	_____
Procedure 3	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Procedure 4	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____

Approximately how much money did you spend on car parking for these visit(s) during [event-name][month]?

Parking Costs ($): Procedure 1	_____ ($)
Parking Costs ($): Procedure 2	_____ ($)
Parking Costs ($): Procedure 3	_____ ($)
Parking Costs ($): Procedure 4	_____ ($)

Approximately how much money did you spend on public transport for these visit(s) during [event-name][month]?

Public Transport Costs ($): Procedure 1	_____ ($)
Public Transport Costs ($): Procedure 2	_____ ($)
Public Transport Costs ($): Procedure 3	_____ ($)

Public Transport Costs ($): Procedure 4

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a melanoma procedure visit in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Procedure 1

_____

Finish Place: Procedure 1

_____

Start Place: Procedure 2

_____

Finish Place: Procedure 2

_____

Start Place: Procedure 3

_____

Finish Place: Procedure 3

_____

Start Place: Procedure 4

_____

Finish Place: Procedure 4

_____

Diagnostic Test or Scans

Did you have any diagnostic tests or scans (e.g., ultrasound, x-ray, computed tomography (CT) scan, magnetic resonance imaging (MRI), position emission tomography (PET), genetic or blood test) because of treatment or follow-up of your melanoma or other skin cancer during [event-name][month]?

◯ Yes
◯ No

How many times did you have diagnostic tests or scans because of treatment or follow-up for your melanoma or other skin cancer during [event-name][month]?

_____

Please select the date of diagnostic test or scan 1

_____

Please select the date of diagnostic test or scan 2

_____

What type of test or scan did you have at these visit(s)?

Diagnostic visit 1	◯ Ultrasound ◯ X-ray ◯ Computed Tomography (CT) scan ◯ Magnetic resonance imaging (MRI) ◯ Position emission tomography (PET) ◯ Genetic test ◯ Blood test ◯ Other
Please specify	_____
Diagnostic visit 2	◯ Ultrasound ◯ X-ray ◯ Computed Tomography (CT) scan ◯ Magnetic resonance imaging (MRI) ◯ Position emission tomography (PET) ◯ Genetic test ◯ Blood test ◯ Other
Please specify	_____

Did you use any of the following modes of transport to travel to these visit(s)? (please select all that apply)

Diagnostic visit 1	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____
Diagnostic visit 2	☐ Car ☐ Bus ☐ Train ☐ Bike ☐ Walking ☐ Other
Please specify	_____

Approximately how much money did you spend on car parking for these visit(s) during [event-name][month]?

Parking Costs ($): Diagnostic visit 1	_____ ($)
Parking Costs ($): Diagnostic visit 2	_____ ($)

Approximately how much money did you spend on public transport for these visit(s) during [event-name][month]?

Public Transport Costs ($): Diagnostic visit 1

_____
($)

Public Transport Costs ($): Diagnostic visit 2

_____
($)

Please list the starting and finishing place(s) (i.e., suburb) of all travel related to these visit(s).

(For example, if you drove from your home in Belrose to a diagnostic test or scan in North Sydney, enter Belrose as the Start Place and North Sydney as the Finish Place)

Start Place: Diagnostic visit 1

_____

Finish Place: Diagnostic visit 1

_____

Start Place: Diagnostic visit 2

_____

Finish Place: Diagnostic visit 2

_____

Other Expenses

Please record all out of pocket expenses you have had during [event-name][month] because of your melanoma or other skin cancer related treatment or follow-up, including melanoma clinical visits, imaging or blood tests, biopsies, surgery sunscreen, sun protective clothing (hat, long sleeved swim shirt etc.), bandages and tape or other medical goods, accommodation to attend melanoma clinic, any extra costs associated with using Teledermatology)

Brief description (1)

_____

Cost (1)

_____
($)

Brief description (2)

_____

Cost (2)

_____
($)

Brief description (3)

_____

Cost (3)

_____
($)

Brief Description (4)

_____

Cost (4)

_____
($)

Brief Description (5)

_____

Cost (5)

_____
($)

Teledermatology Use

Did you take any images of your target lesion or other lesion(s) of concern with your MoleScope device during [event-name][month]?	◯ Yes ◯ No
How many hours in total did you spend using your MoleScope device during [event-name][month]? This includes time spent taking images, submitting them and reviewing teledermatology reports	_____ (hours)

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